AMERICAN PSYCHIATRIC ASSOCIATION

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2025 Billing and Documentation Update for Psychiatrists

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The faculty report no financial relationships or conflicts of interest to disclose.

Review the CPT 2025 manual and check with your local Medicare carrier and/or commercial payers for updates and any payer-specific billing/coding guidelines.

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OVERVIEW

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- **Review** new/revised billing codes, including telehealth
- **Highlight** other codes of interest
- **Review** Office/Outpatient E/M code patterns
- **Review** generally accepted documentation requirements for billing psychiatric services

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OVERVIEW OF KEY CHANGES FOR 2025

NEW/REVISED CODES FOR 2025

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- Telehealth Services
- Brief Synchronous Communication Technology Evaluation and Management Service (eg, Virtual Check-In)
- Safety Planning Interventions
- Digital Mental Health Treatment Devices

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TELEHEALTH

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- CPT created 17 new CPT codes to report telehealth services.
- **CMS DID NOT adopt** those new codes; they directed clinicians to (continue) to use the existing outpatient E/M codes (99202-99205 and 99212-99215) to report telehealth care (both Audio/Video and Audio-Only) in the Outpatient/Office setting.
- **Private payers are divided** as to which codes to use. Some have followed Medicare; others have adopted the CPT codes. Confirm what billing codes (and modifiers) are required when billing private payers or Medicaid.

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AUDIO/VIDEO CODES

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New CPT Code	Level of MDM	Time Threshold*	New/Established Patient	Analogous To
98000	Straightforward	15	New	99202
98001	Low	30	New	99203
98002	Moderate	45	New	99204
98003	High	60	New	99205
98004	Straightforward	10	Established	99212
98005	Low	20	Established	99213
98006	Moderate	30	Established	99214
98007	High	40	Established	99215

*Total Time must be met or exceeded

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AUDIO-ONLY CODES

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New CPT Code	Level of MDM	Time Threshold*	New/Established Patient	Analogous To
98008	Straightforward	15	New	99202
98009	Low	30	New	99203
98010	Moderate	45	New	99204
98011	High	60	New	99205
98012	Straightforward	10	Established	99212
98013	Low	20	Established	99213
98014	Moderate	30	Established	99214
98015	High	40	Established	99215

*Total Time must be met or exceeded

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TELEHEALTH: OTHER PSYCHIATRIC SERVICES

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- CMS and the CPT Manual update the listing of telehealth eligible services annually. The following is a list of the most commonly billed services currently on the list.

Code	Short Descriptor	Finalized Action	Code	Short Descriptor	Finalized Action
90785	Psytx complex interactive	permanent	90838	Psytx w pt w e/m 60 min	permanent
90791	Psych diagnostic evaluation	permanent	90839	Psytx crisis initial 60 min	permanent
90792	Psych diag eval w/med srvcs	permanent	90840	Psytx crisis ea addl 30 min	permanent
90832	Psytx w pt 30 minutes	permanent	90845	Psychoanalysis	permanent
90833	Psytx w pt w e/m 30 min	permanent	90846	Family psytx w/o pt 50 min	permanent
90834	Psytx w pt 45 minutes	permanent	90847	Family psytx w/pt 50 min	permanent
90836	Psytx w pt w e/m 45 min	permanent	90853	Group psychotherapy	permanent
90837	Psytx w pt 60 minutes	permanent	90875	Psychophysiological therapy	provisional

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TELEHEALTH: OUTPATIENT EVALUATION AND MANAGEMENT SERVICES

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- The Medicare requirement that a patient must be seen in-person within six months of providing a telehealth visit and annually thereafter has been waived through March 31, 2025.
- The location of a patient during telehealth may include: Patient's home, Homeless shelters, Group homes, Hotels, Other settings that the beneficiary identifies as home or residence, whether permanent or temporary. Home also include circumstances where the patient, for privacy or other personal reasons, chooses to travel a short distance from their home for a telehealth service.
- For care provided to Medicare-covered patients in their homes (Place of Service 10):
  - No modifier is required for audio/video care
  - Modifier 93 is required for audio-only care
  - Modifier FQ is required for audio-only care in FQHCs/RHCs
- Private payer and Medicaid requirements regarding the use of modifiers vary.

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TELEHEALTH: OUTPATIENT EVALUATION AND MANAGEMENT SERVICES

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- Documentation should include the modality used to provide care.
- Medicare allows the use of audio-only care if the **patient** does not have access to or the ability to use an audio/video option; this should be documented in the record.
- According to CPT: If the audio-video connection is lost during the visit and only audio is restored, report the service that accounted for the majority of the time of the interactive portion of the service. Ten minutes of medical discussion or patient observation must be exceeded in order to report the audio-only service.

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BRIEF SYNCHRONOUS COMMUNICATION TECHNOLOGY EVALUATION AND MANAGEMENT SERVICE (EG, VIRTUAL CHECK-IN)

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Code	Description
98016	Brief communication technology-based service (eg, virtual check-in) by a physician or other qualified health care professional who can report evaluation and management services, provided to an established patient, not originating from a related evaluation and management service provided within the previous 7 days nor leading to an evaluation and management service or procedure within the next 24 hours or soonest available appointment, 5-10 minutes of medical discussion

Do not report 98016 in conjunction with 98000-98015
Do not report services of less than 5 minutes of medical discussion

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BRIEF SYNCHRONOUS COMMUNICATION TECHNOLOGY EVALUATION AND MANAGEMENT SERVICE (EG, VIRTUAL CHECK-IN)

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- 98016 replaces the CMS “virtual check-in” visit, HCPCS G2012
- It describes services for **established patients** with 5-10 minutes of medical discussion and not MDM. Do not count time for establishing the connection or arranging the appointment and services of less than five minutes are not billed.
- The service is **patient-initiated,** and video technology is not required. The service is seen as a way to determine if a different type of visit (i.e. E/M) is needed
- When the call leads to an E/M service on the same day, and when time is used to select the level of that E/M service, the time from 98016 may be added to the time of the E/M service.

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NEW MEDICARE COVERAGE FOR SPI AND DMHT

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Safety Planning Interventions (SPI)	Digital Mental Health Treatment Devices
Medicare coverage for evidence-based SPI, including office-based practices. Currently no formal guidance from Medicare re documentation. • Safety Planning Intervention code (G0560) - Must be done **personally** by the billing clinician - Can be billed in addition to E/M or therapy - Billed in 20-minute units • Post-Discharge Telephonic Follow-up Contacts Intervention (G0544) - Typically done during transition from ED or other crisis encounter when indicated - Includes 4 calls per month (at least 1 real-time telephone interaction required) - Requires patient consent.	Medicare coverage for costs associated with the use of FDA-approved digital mental health treatment (DMHT) devices purchased by the clinician. Currently no formal guidance from Medicare re documentation. • Supply of the DMHT device and initial education and onboarding (G0552) • Monthly treatment management services directly related to the patient's therapeutic use of the device, first 20 minutes* (G0553) - Each additional 20 mins* (G0554) *Minutes accumulated over the course of a calendar month.

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OTHER CODES TO KNOW

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- Visit complexity code (G2211)
- Online Digital Evaluation and Management Services (99421-99423)
- Interprofessional Telephone/Internet/Electronic Health Record Consultations (99446-99449 and 99451-99452)
- Multiple-family group behavior management/modification training for parent(s)/guardian(s)/caregiver(s) of patients (96202-96203) (i.e. group caregiver training)

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VISIT COMPLEXITY CODE (G2211)

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HCPCS Code	Description
G2211	[Visit complexity inherent to evaluation and management associated with medical care services that serve as the continuing focal point for all needed health care services and/or with medical care services that are part of ongoing care related to a patient's single, serious condition or a complex condition. (add-on code, list separately in addition to office/outpatient evaluation and management visit, new or established)]

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VISIT COMPLEXITY

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Use the add-on code when you are:

- The primary physician caring for all the patient's healthcare needs (like a primary care physician or a psychiatrist caring for the mental health and physical needs of patients with serious mental illness), or
- Providing ongoing care for a single, serious condition or a complex condition.

Do not use the add-on code when:

- Your relationship with the patient is of a discrete, routine, or time-limited nature.
- The associated office visit E/M is reported with modifier 25.
- Reporting CPT code 99211.

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EXAMPLES TO CONSIDER FOR G2211

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Continuing focal point for all health care services

- Patient being seen for longitudinal follow-up for schizophrenia and hypertension. The patient has not established care with a PCP given factors related to the patient's psychiatric diagnosis and/or social determinates of health, so you have started prescribing antihypertensive medications to reduce risk.

Ongoing care for a single serious condition or complex condition

- Patient being seen for longitudinal follow-up of schizophrenia or schizoaffective disorder.
- Patient being seen for longitudinal follow-up of moderate or severe intellectual disability.
- Patient being seen for longitudinal follow-up of a serious personality disorder.
- Patient being seen for longitudinal follow-up of a severe substance use disorder.

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ONLINE DIGITAL EVALUATION AND MANAGEMENT SERVICES

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CPT Code	Description
99421	Online digital evaluation and management service, for an established patient, for up to 7 days, cumulative time during the 7 days; 5-10 minutes
99422	11-20 minutes
99423	21 or more minutes

- For patient-initiated services through HIPAA compliant platforms for established patients.
- These are not for the nonevaluative electronic communication of test results, scheduling of appointments, or other communication that does not include E/M.
- Online digital E/M services require permanent documentation storage (electronic or hard copy) of the encounter.
- Check the CPT for reporting requirements when the service occurs within 7 days of an E/M visit.

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INTERPROFESSIONAL TELEPHONE/INTERNET/ELECTRONIC HEALTH RECORD CONSULTATIONS

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CPT Code	Description
99446	Interprofessional telephone/Internet/electronic health record assessment and management service provided by a consultative physician or other qualified health care professional, **including a verbal and written report** to the patient's treating/requesting physician or other qualified health care professional; 5-10 minutes of medical consultative discussion and review
99447	11-20 minutes of medical consultative discussion and review
99448	21-30 minutes of medical consultative discussion and review
99449	31 minutes or more of medical consultative discussion and review
99451	Interprofessional telephone/Internet/electronic health record assessment and management service provided by a consultative physician or other qualified health care professional**, including a written report to the patient's treating/requesting physician or other qualified health care professional**, 5 minutes or more of medical consultative time
99452	Interprofessional telephone/Internet/electronic health record **referral service(s) provided by a treating/requesting physician** or other qualified health care professional, 30 minutes

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INTERPROFESSIONAL TELEPHONE/INTERNET/ELECTRONIC HEALTH RECORD CONSULTATIONS

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- The patient may be either a new patient to the consultant or an established patient with a new problem or an exacerbation of an existing problem.
- The consultant should not have seen the patient in a face-to-face encounter within the last 14 days.
- When the consultation leads to a transfer of care or other face-to-face service within the next 14 days or next available appointment date of the consultant, these codes are not reported.
- The majority of the time reported must be devoted to the verbal or Internet discussion.
- If more than 50% of the time is spent on data review and/or analysis these codes (99446-99449) should not be billed.
  - 99451 can be billed in this circumstance

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GROUP CAREGIVER TRAINING

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CPT Code	Description
96202	Multiple-family group behavior management/ modification training for parent(s)/guardian(s)/ caregiver(s) of patients with a mental or physical health diagnosis, administered by physician or other qualified health care professional (without the patient present), face-to-face with multiple sets of parent(s)/guardian(s)/ caregiver(s); initial 60 minutes
96203	-each additional 15 minutes (List separately in addition to code for primary service)

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GROUP CAREGIVER TRAINING

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- Purpose of the codes is to teach the parent(s)/ guardian(s)/caregiver(s) interventions that they can independently use to effectively manage the identified patient's illness(es) or disease(s).
- These skills and strategies help to support compliance with the identified patient's treatment and the clinical plan of care.
- Codes are used to report the total duration of face-to-face time spent by the physician or other qualified health care professional providing the services.
- For counseling and education provided by a physician or other qualified health care professional to a patient and/or family, see the appropriate evaluation and management codes, including office or other outpatient services.

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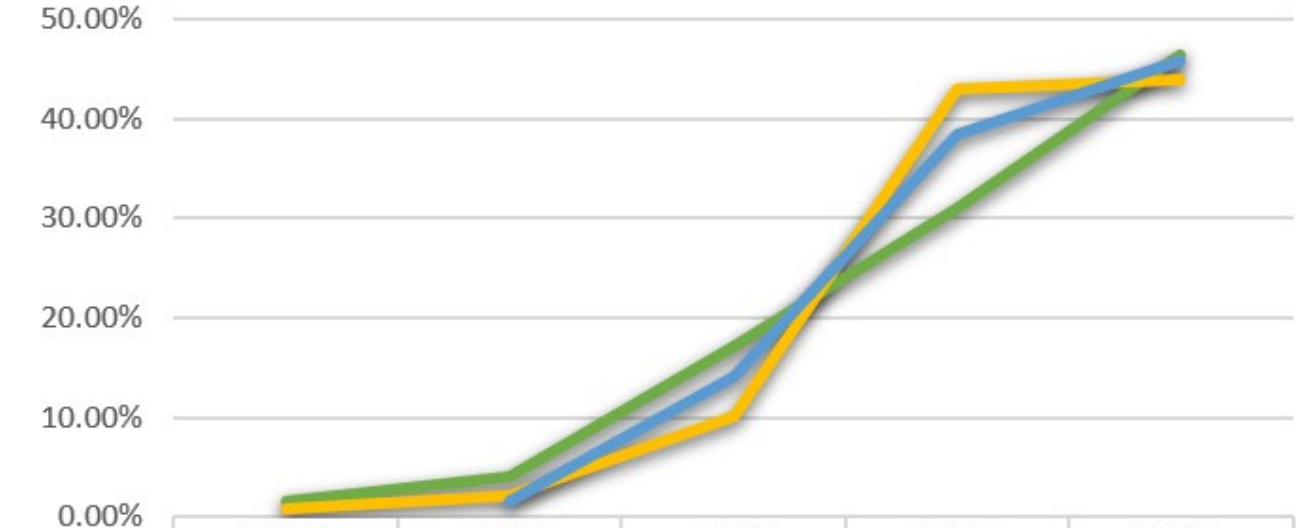
Overview General Outpatient E/M Coding Patterns

E/M BILLING PATTERNS BASED ON MEDICARE DATA



	99201	99202	99203	99204	99205
2011	1.49%	3.96%	17.11%	31.02%	46.41%
2017	0.77%	2.09%	10.12%	43.02%	44.00%
2023		1.54%	14.18%	38.42%	45.86%

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E/M BILLING PATTERNS BASED ON MEDICARE DATA (CONTINUED)

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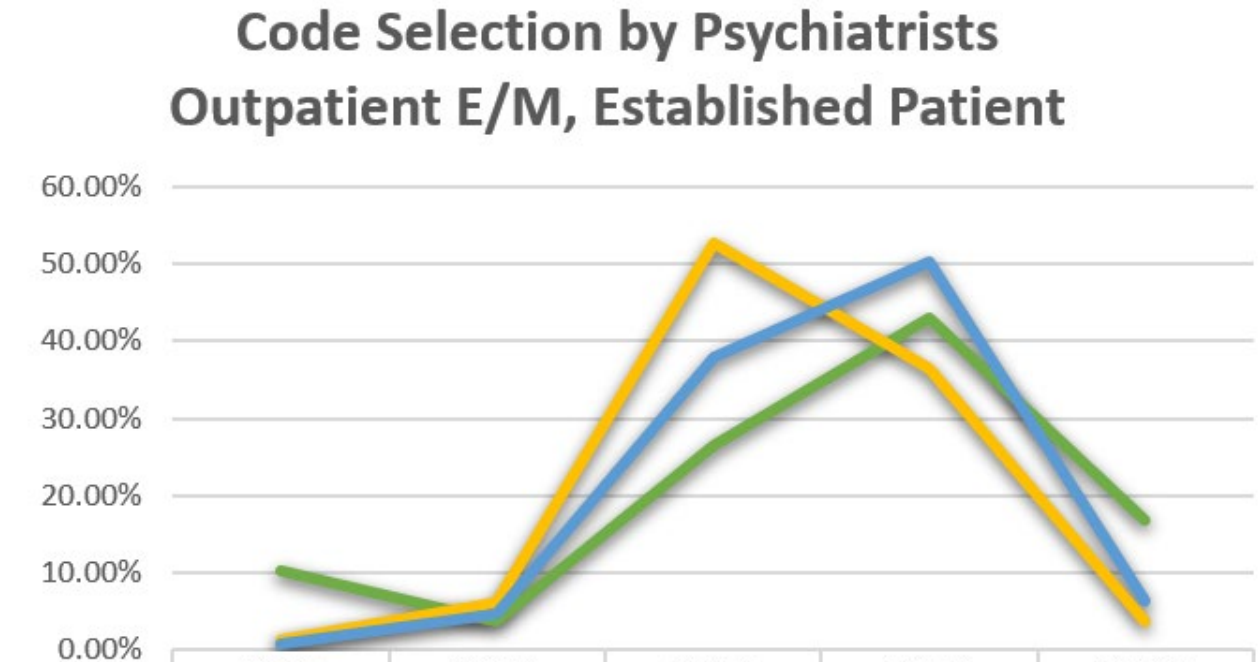

	99211	99212	99213	99214	99215
2011	10.26%	3.64%	26.40%	42.95%	16.75%
2017	1.22%	6.00%	52.65%	36.40%	3.72%
2023	0.72%	4.64%	37.93%	50.25%	6.45%

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General Documentation Rules

DOCUMENTATION TABLE

Documentation Element	Clinical Reasons	Billing Reasons
Reason for Visit/Chief Complaint	X	X
History of Present Illness	X	
Review of Systems	X	
Past Psychiatric History	X	
Family Psychiatric History	X	
Past Medical History	X	
Mental Status Exam	X	
Assessment/Plan (MDM)	X	X
Time-based Billing		X

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DOCUMENTING PSYCHOTHERAPY ADD-ON SERVICE

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- Should be separate from and follow the documentation related to the E/M visit. To Include:
  - Time spent providing psychotherapy (requirements by payer vary; some payers/states may require start/stop times)
  - Type of therapeutic intervention (i.e., insight oriented, supportive, behavior modification)
  - Target symptoms
  - Progress toward goals

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EXAMPLES

OUTPATIENT EXAMPLE – 99213

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CC: Follow-up visit for a patient with recurrent major depressive disorder, in remission.

Hx: Margaret tells me that she has been doing pretty well since our last visit 3 months ago. Her PCP has not changed her HCTZ blood pressure medicine. She has been sleeping and eating well, her return to work has been good for her, and she is functioning well in that setting. **She has resumed her usual activities and has had no suicidal ideation nor periods lasting more than a few minutes of sadness throughout the day. We agreed to continue the fluoxetine at 40 mg.**

Exam: Well dressed, euthymic affect, normal speech. Denies suicidal/homicidal thoughts, intent, or plan.

MDM: **Stabilizing mood in patient with recurrent episodes of depression. Will continue fluoxetine** 40 mg daily. Rx given for 3 months. Return in 3 months or PRN.

Dx: **Major depressive disorder, recurrent**.

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OUTPATIENT EXAMPLE - 99214

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CC: Follow-up medication management visit for 54 yr old man with hx of bipolar disorder and alcohol use disorder last seen 3 months ago.

Hx: Tom states that he has been doing well on his current regimen of medications. He has been sticking to a stable routine, as we previously discussed. His work has been good, and his relationship with his wife is improving although it remains "bumpy." He is getting at least 6 hours of sleep at night and works hard at going to bed and getting up at the same times. He has had none of the early signs of an impending manic episode that we have discussed. **He proudly tells me he is now about to celebrate 2 years of sobriety**. **He continues to attend regular Alcoholics Anonymous meetings and stays in regular contact with his sponsor.**

Exam: Appropriately dressed and groomed male with full affect and stable mood. **No evidence of hypomania or depression and no suicidal ideation or intentions.** Speech is normal rate and rhythm. No evidence of psychosis.

MDM: Tom remains stable on his current medication regimen. Discussed possible return to marital therapy, but he wants to think about it before restarting. Upcoming 2-year anniversary of sobriety.

Dx: **Bipolar I disorder, in partial remission**; **alcohol use disorder**.

Plan: **Continue with lamotrigine 200 mg daily**. **Continue with Alcoholics Anonymous** meetings and sponsor's support. Will assist with return to marital therapy if patient and wife agree. Next appointment in 3 months or PRN.

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PSYCHOTHERAPY

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Psychotherapy note should be separate and distinct from E/M documentation appearing after the content related to E/M

General Guidelines for Psychotherapy Note

Date: March 20, 2024

Modality/Therapeutic intervention: Psychodynamic

Goals: Reduction in depressive symptoms and improved relationship with husband.

Progress toward achievement of treatment goals: Depressive symptoms gradually improving. Has been more aware of husband's perspective during recent arguments, which has helped to reduce escalation of arguments.

Time spent providing psychotherapy (requirements by payer vary; some payers/states may require start/stop times)

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Questions

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Resources

APA RESOURCES

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APA CPT Coding and Reimbursement
https://www.psychiatry.org/Psychiatrists/Practice/Practice-Management/Coding-and-Reimbursement

APA Practice Management Helpline
https://www.psychiatry.org/psychiatrists/practice/practice-management/practice-management-helpline

CMS Medicare Learning Network (MLN) Booklet: Medicare & Mental Health Coverage
https://www.cms.gov/files/document/mln1986542-medicare-mental-health-coverage.pdf

CMS MLN Booklet: Evaluation and Management Services Guide
https://www.cms.gov/outreach-and-education/medicare-learning-network-mln/mlnproducts/downloads/eval-mgmt-serv-guide-icn006764.pdf

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